



Name : SAROJ - AGARWAL
Age/Sex : 63 Yrs. Female
Patient Id : 59568/SD
Treating Dr. : Dr. SANJAY AGRAWAL

Date : 06-Jul-2026 1:31 pm
Rec. No : 5796
Indoor No : I/0726/907
Ref. By Dr. : DR. SANJAY AGRAWAL

Iron Profile

TEST	RESULTS	UNIT	REFERENCE RANGE
Iron	55.7	µg/dL	Male : 59 - 158 Female : 37 - 145
TOTAL IRON BINDING CAPACITY (TIBC)	310	µg/dl	Male: 250 - 450 µg/dl Female: 250 - 450 µg/dl
% TRANSFERRIN SATURATION	17.96	%	13 - 45 %

INTERPRETATION : Iron determinands are performed for the diagnosis & monitoring of treatment of all the microcytic hypochromic diseases such as Iron deficiency anemia, hemochromatosis, chronic renal diseases as well as macrocytic anemia & also normocytic anemia, hemolytic anemia, hemoglobinopathies, bone marrow diseases & toxic bone marrow damage interpretation of iron status must be correlated with other parameters given below as a whole studies rather than interpreting a single test.

1. IRON DEFICIENCY ANEMIA: S.Iron (Low), TIBC(High), Transferrin saturation(low), Ferritin(low), HB Electrophoresis (Normal), Smear(Micro/hypo)
2. ANEMIA OF CHRONIC DISEASES: S.Iron (Low), TIBC(low), Transferrin saturation(low), Ferritin(Normal/High), HB Electrophoresis (Normal), Smear(Micro/hypo/Normal)
3. THALASSEMIA : S.Iron (Normal/High), TIBC(Normal), Transferrin saturation(Normal/High), Ferritin(High), HB Electrophoresis Abnormal, Smear(Micro/hypo with target cells)
4. SIDEROBLASTIC ANEMIA : S.Iron (Normal/High), TIBC(Normal/High), Transferrin saturation(Normal/High), Ferritin(High), HB Electrophoresis Normal, Smear (Micro/hypo/Variable)
5. IRON OVERLOAD (HEMOCHROMATOSIS): S.Iron (High), TIBC(low), Transferrin saturation(High), Ferritin(High), HB Electrophoresis Normal, Smear(Variable)
6. MEGALOBlastic ANEMIA : S.Iron (High), TIBC(low), Transferrin saturation(High), Ferritin(High), HB Electrophoresis Normal, Smear(Macrocytic) (Harrison's principles of internal medicine vol - 1, 15th edition/663)

Verified By

Operator Name : Amrita

Emergency 24 x 7 & 365 Days.

Print Date & Time : 06-Jul-2026 02:28:57PM

Corroborative Clinicopathologic Correlation is Essential for Final diagnosis. Test reports are subject to technical limitation & should be Clinically Co-related. Laboratory may be Contacted if Required. Emergency 24 Hours.

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Maitreya Medicare Limited

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Dr. Jitendra Kumar V Dubey MBBS MD
(Pathology)
Consultant Pathologist (GRN No. 15107)



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FERRITIN ESTIMATION

TEST	RESULTS	UNIT	REFERENCE RANGE
FERRITIN LEVEL			
FERRITIN, Serum	<u>60.7</u>	µg/L	MALE(>5 yrs.): 15-200 FEMALE(>5 yrs.): 15-150 Children(<5 Years) : 12-200

Note: Increase in serum ferritin due to inflammatory conditions (Acute phase response) can mask a diagnostically low result.

Introduction : Ferritin is a high-molecular weight iron-containing protein that functions in the body as an iron storage compound. It has been demonstrated that the Ferritin molecule, when fully saturated, may consist of over 20% iron by weight. Approximately 25% of the iron in a normal adult is present in various storage forms. About two-thirds of the iron stores in the human body exist in the form of Ferritin.

Use : Diagnosis of iron deficiency or excess; correlates with total body iron stores

?Predict and monitor iron deficiency?Determine response to iron therapy or compliance with treatment.

?Differentiate iron deficiency from chronic disease as cause of anaemia.?Monitor iron status in patients with chronic renal disease with or without dialysis.

?Detect iron overload states and monitor rate of iron accumulation and response to iron depletion therapy.?Population studies of iron levels and response to iron supplement.

Clinical Significance :

Decreased in: Iron Deficiency anaemia

Increased in: Ferritin is an acute-phase increased in many patients with various acute and chronic liver diseases, alcoholism, infection and inflammation, hyperthyroidism, Gaucher disease, acute myocardial infarction, iron overload, Renal cell carcinoma, Serum Ferritin may not be decreased when iron deficiency coexists with these conditions. Variations due to age Increases: with age, higher in men than women, in women on OCP, in person who eat red meat compared to vegetarians DILUTION PROTOCOL: (on request) At our lab with kit, manual dilution protocol has been validated for FERRITIN up to 1:20 dilution and result up to 40000 NG/ML. After above dilution, it will be done manually and because of Ag-Ab reaction curve it may be erroneous if diluted after validated dilution.

Adv.: Kindly Correlate Clinically

Verified By

Dr. Jitendrakumar V. Dubey MBBS MD
(Pathology)

Consultant Pathologist (GRN No. 35107)

Operator Name : Amrita

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